

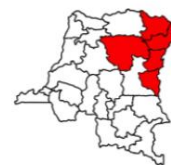


Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°088/MVEBDB/10/08/2026

Reporting date: August 10, 2026

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**HIGHLIGHTS** (last 24 hours)

In the past 24 hours, 68 new confirmed cases and 50 deaths (31 community deaths and 19 intra-ETC) of Ebola Virus Disease (EVD) have been reported in the provinces of Ituri (53 cases), North Kivu (15 cases).

ACCUMULATIVE CASE 4,449	ACCUMULATIVE CONFIRMED DEATHS 2061	LETHALITY 46.3%	PATIENTS IN ISOLATION / CTE 716	ACCUMULATIVE HEALED 886	TRACKING RATES CONTACTS 82.7% (Of the Day)
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Affected Provinces and Affected Health Zones

**5 Provinces Affected**

Haut-Uélé, Ituri, North Kivu,
South Kivu & Tshopo

**53 Health Zones**

Affected Out of
140 (37.9%)



246 Health areas
affected

Out of 3104 (7.9%)

Summary of key points from the last 24 hours

Over the past 24 hours, 68 new confirmed cases and 50 deaths (31 community deaths and 19 intra-ETC deaths) of Ebola Virus Disease (EVD) have been reported in the provinces of **Ituri (53 cases) and North Kivu (15 cases)**, reflecting continued active transmission, primarily in Ituri. Haut-Uélé has not transmitted any data.

To date, the Democratic Republic of Congo has recorded 4,449 confirmed cases, including 2,061 deaths, representing a case fatality rate of 46.3%. Ituri remains the epicenter of the epidemic, accounting for 85.7% of cumulative confirmed cases and 77.9% of new cases reported in the last 24 hours.

Seventeen patients were declared recovered after receiving two negative Ebola virus (EV) test results (16 in Ituri and 1 in North Kivu). Conversely, 19 deaths within the Ebola treatment center were recorded in Ituri (12) and North Kivu (7).

The epidemic affects 53 of the 140 health zones in the five affected provinces. Ituri has 28 out of 36 health zones affected, followed by North Kivu (12/34), and the Haut-Uélé (6/13), Tshopo (6/23) and South Kivu (1/34).

Distribution of confirmed cases and deaths by affected province

(As of August 10, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	3,814	1,653	43.3%	28/36 (77.8%)	53
North Kivu	510	354	69.4%	12/34 (35.3%)	15
Haut-Uélé	113	48	42.5%	6/13 (46.1%)	ND
Tshopo	9	5	55.6%	6/23 (26.0%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	4,449	2061	46.3%	53/140 (37.9%)	68

Confirmed cases and deaths by province and health zone (situation as of August 10, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within CTE	Total confirmed deaths
Ituri	3814	1653	43.3%	53	21	12	33
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	7	2	28.6%				0
Aungba	10	5	50.0%				0
Bamboo	93	22	23.7%	1	0		0
Boga	1	1	100.0%				0
Bunia	1043	317	30.4%	22	7		7
Damascus	18	8	44.4%				0
Drodro	12	6	50.0%				0
Fataki	55	29	52.7%	1	0		0
Gethy	2	1	50.0%				0
Kambala	2	0	0.0%				0
Kilo	29	11	37.9%				0
Komanda	42	29	69.0%				0
Lita	163	96	58.9%				0
Logo	11	5	45.5%				0
Lolwa	11	4	36.4%				0
Mahagi	4	1	25.0%				0
Mambasa	13	6	46.2%				0
Mandima	19	13	68.4%				0
Mangala	116	74	63.8%				0
Mongbwalu	576	280	48.6%	10	6		6
Nia-Nia	142	84	59.2%	4	2		2
Nizi	489	227	46.4%	10	4		4
Nyankunde	116	35	30.2%				0
Rimba	9	4	44.4%				0
Rwampara	765	303	39.6%	5	2		2
Tchomia	48	24	50.0%				0
To be ventilated	N / A	59*	N / A			12	12
North Kivu	510	354	69.4%	15	10	7	17
Blessed	78	60	76.9%	2	2	1	3
Butembo	103	85	82.5%	2	2		3
Goma	1	0	0.0%				0
Kalunguta	10	4	40.0%				0
Katwa	239	156	65.3%	10	5	5	10
Kyondo	13	10	76.9%				0

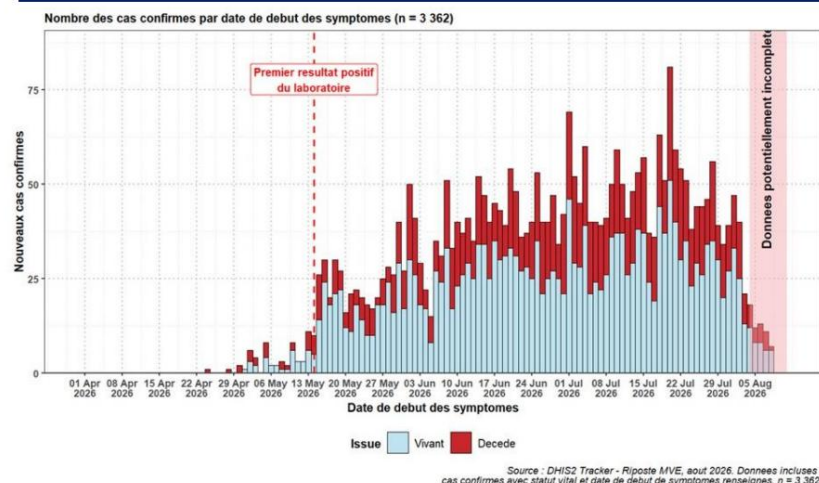
Lubero	1	1	100.0%				0
Mabalako	1	0	0.0%				0
Masereka	7	4	57.1%				0
Musienene	49	29	59.2%			1	1
Oicha	5	4	80.0%				0
Vuhovi	3	1	33.3%				0
Haut-Uélé	113	48	42.5%	ND	ND	ND	0
Boma Mangbetu	21	5	23.8%	ND	ND	ND	0
Gombari	1	1	100.0%	ND	ND	ND	0
Isiro	31	16	51.6%	ND	ND	ND	0
Pawa	17	11	64.7%	ND	ND	ND	0
Rungu	1	0	0.0%	ND	ND	ND	0
Wamba	42	15	35.7%	ND	ND	ND	0
Tshopo	9	5	55.6%	0	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	2	1	50.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	3	3	100.0%				0
Mangobo	1	1	100.0%				0
Wanie-Rukula	1	0	0.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Total	4449	2061	46.3%	68	31	19	50

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

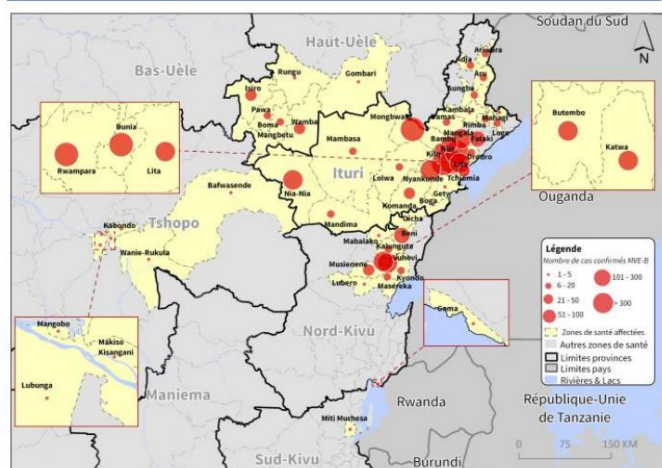
Status of alerts notified by province, as of August 10, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	ND	ND	ND	ND	ND	ND	ND	ND	ND
Ituri	525	79	604	150	79	259	0	229	149
North Kivu	832	72	904	81	72	751	0	153	66
South Kivu	8	0	8	1	0	7	0	1	1
Tshopo	88	1	89	5	1	82	0	6	5
Total General	1453	152	1,605	237	152	1099	0	389	221

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : Support for the Beni zonal coordination and continuation of the mission support for the Butembo health zone.

1.2. Epidemiological surveillance

1.2.1. Main Actions

- A total of 1,605 alerts were recorded, of which 1,488 (92.7%) were verified in the four provinces that submitted data for August 10, 2026. However, 389 (26.1%) were validated as suspected cases and all were investigated, of which 221 Suspects transferred to CT/CTE.
- The proportion of contacts followed up in the last 24 hours is at 82.7% (13,951 seen out of 16,862 to be followed up), close to the threshold of 85%, with lower performance in North Kivu (74.7%).

1.2.2. Challenges

Low reporting of alerts and listing and monitoring of contacts around confirmed cases.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- Five alerts were detected at the various PoE/PoCs (3 in Ituri and 2 in North Kivu): the three live alerts in Ituri were validated, including one confirmed positive at the Pluto PoC (Mongbwalu Health Zone), and two referred to the CTE; in North Kivu, the two live alerts notified at the Mukulia PoC were isolated, validated, and referred to the Kanzuli CT. No bodies were intercepted today.
- In total, 216,357 travelers were screened (135,971 in Ituri, 76,218 in North Kivu, and 4,168 in Tshopo), with a screening rate of 97%. In Ituri, only one of the 11 strategic PoE/PoC sites (9.1%) operated without interruption, and the completeness rate reached 58/60 (96.6%); in North Kivu, all 18 sites reported (100%); in Tshopo, 6 out of 19 PoC sites were activated (32%).

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: functionality remains low in Tshopo, with only 6 out of 19 (32%) activated, and reporting completeness is limited to 57%.

- Constraints affecting PoE/PoC functionality: performance remains affected by refusals to screen (2,018 people, or 2.6%, in North Kivu), attacks on service providers and non-compliance with health protocol by some authorities at the Mukulia PoC (North Kivu), as well as by the lack of means of transport for supervision in Tshopo.

1.4. Laboratory

1.4.1. Main actions

- Ituri recorded **63** positive results, including **53 new cases** (**32** living and **21** deaths) and **10** repeat tests, while **North Kivu** confirmed **15 cases, including 10 deaths**. In total, **376** samples were analyzed, confirming **78 new cases** of EVD (positivity: **20.7 %**).

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- In Ituri, 23/76 rings were opened; 55/55 sites identified and decontaminated (100%) including 44 households and 11 health facilities; 70 EDS carried out on 92 death alerts (76.1%) and 52 bodies collected post-mortem.
- In North Kivu, 222 health facilities were supported and 767 providers were briefed; 7 facilities were evaluated by scorecard (average score 40.9%), 54 EDS were carried out and 94 bodies were collected and secured.
- In Tshopo, IPC activities were carried out in 4 health zones (4 rings opened, 4 households and 2 establishments decontaminated) and a lifeless body was prepared for EDS at the HC of Kisangani.

1.5.2. Challenges

- Weak implementation of IPC standards: partial decontamination of households of cases in Ituri (43/76 not decontaminated), limited completeness of reporting in North Kivu (7 health zones out of 11) and low card scores (40.9% on average in North Kivu and 45.2% at the Rekapi Polyclinic in Tshopo), demonstrating a continued risk of healthcare-associated infections.

1.6. Continuity of care

1.6.1. Main actions

- In Ituri, 26 out of 30 ambulances allocated to the response were available, with 38 trips completed, 32 of which were successful, evacuating 37 patients. The Ebola Treatment Centers (ETCs) recorded 78 admissions and 63 discharges, including 16 recoveries in the last 24 hours; occupancy reaches 417 patients for 845 beds (49.3%) specifically in Ituri, with saturation of the CTEs of Rwampara, ISTM and Nizi and the CT of Nia-Nia.
- In North Kivu, there have been 215 cumulative admissions to CTE/CT, including 212 confirmed cases, with an intra-CTE mortality rate of 40%; 47 admissions and 1 recovery today while 7 intra-CTE deaths have been reported.

1.6.2. Challenges

Insufficient availability of referral resources (4 ambulances out of service in Ituri, no ambulances in 7 health zones of North Kivu, and no TSE/CT centers in 4 health zones) and saturation of several TSE/CT centers, limiting rapid access to care and isolation of suspected cases. In Haut-Uélé : the lack of standardized TSE centers persists in the 6 affected health zones

1.7. Risk communication and community engagement

1.7.1. Main actions

- In Ituri, support was provided for listing 113 contacts around 8 cases in six health areas of the Rwampara health zone; 5,016 people were reached during home visits and 2,297 during...

Educational talks; 45 feedback sessions collected, 20 of which were clarified (44.4%), and 22 radio programs produced.

- **In North Kivu**, 28 confirmed cases were the subject of communication actions with 158 household heads; 15 rumors were clarified and 11 community incidents were notified to the surveillance pillar.
- In Tshopo, 771 people were reached during home visits in the health zones of Makiso-Kisangani and Mangobo, 60 pressure group leaders were briefed and 54 women were sensitized in Kabondo.

1.7.2. Challenges

Persistence of community challenges: announced strike of the psychosocial pillar and demotivation linked to non-payment in North Kivu, removal of a positive body by the community at the family cemetery of AS Misebere (ZS Katwa), refusal of dignified and secure burials (10 failures in Mangala and 4 in Komanda in Ituri) and refusal of collection in Tshopo, likely to affect the communities' adherence to the response.

1.8. Logistics

1.8.1. Main actions

In Ituri, the installed capacity reaches 845 beds (417 for confirmed cases, 428 for suspected cases); 189 cartons of F95 masks were received and 500 liters of fuel were distributed; 49 vehicles including ambulances are allocated to the response; the expansion of the CTEs in Bambu, Nizi and ISTM is requested in the face of saturation.

In North Kivu : reception and distribution of 1,200 liters of diesel and delivery of PCI inputs; continuation of the construction of the tank bases at the PoCs of Karuruma and Komba and of the triage unit of the Vuhovi General Referral Hospital (ULB Cooperation); packaging of nutritional inputs for seven health zones).

1.8.2. Challenges

Persistence of logistical needs to support the expansion of the response, including the completion of CTE extensions, the continued supply of essential inputs and PPE, and the availability of means of transport and evacuation.

1.9. Security 1.9.1.

Main actions

- In North Kivu, eight public order maintenance and restoration activities were carried out, including accompanying two burials of confirmed cases, securing the transfer of bodies to the morgues of Ngilika and Katwa and supporting swab operations.

1.9.2. Challenges

- Persistent insecurity, incidents involving local authorities and community resistance during dignified and secure burials, limiting access for response teams: physical assaults on service providers and law enforcement at the PoC in Mukulia, insecurity and broken suits blocking 10 EDS in Mangala.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Continued awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA) in the various communities of the affected provinces.

1.10.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse is still insufficient in all areas concerned by the response.

KEY MESSAGES

Ebola transmission remains active, with 68 new cases and 50 deaths, including 31 fatalities.

community and 19 intra-CTE recorded in the last 24 hours, bringing the national total to 4,449 confirmed cases and 2,061 deaths.

Ituri remains the epicenter of the epidemic, concentrating 85.7% of cumulative confirmed cases and 77.9% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).

Surveillance has performed well over the past 24 hours, with 100% of suspected cases investigated, a follow-up rate of 82.7% close to the threshold of 85.0%.

Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of coordination and support mechanisms.



EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: diedonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. Kombe NGWAMA John

Editorial Team

Professor Dr. Mulangu Katulumba Félix-Médard

Dr. Katusi Biselenge Jules

Mayele Afua

With technical support from the World Health Organization (WHO), Africa CDC.

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd